Defence Q&A preparation: Salama Check-In

Short answers to the questions a panel is most likely to ask. Say them in your own words. Where a figure or name is not yet known, say so plainly and say when it will be confirmed.

Why is this study needed? (relevance)

Strain is common and most people do not seek help. The students who leave are already enrolled, so they are the ones a routine, confidential service can reach. DCHS has no such routine route today. Published figures are from other settings and I say so; this study measures the local picture.

What is the research gap?

I found no official KDF or DCHS figures on suicide, violence at home or debt, and no routine confidential way to see strain early. The gap is both a service gap and a data gap. Media reports are treated as signals, not rates.

Is this a diagnosis tool?

No. It is a screening and support tool. Nothing on screen replaces a clinician. The colour is a prompt to offer help, never a verdict.

What are your objectives, and are they measurable?

Six objectives, each with a measure, a pilot target and a month (Table 1.1 of the proposal). Examples: at least 100 complete check-ins, SUS above 68, and 80% of same-day requests contacted within 8 hours. The targets are pilot targets, confirmed with my supervisor and the steering group.

Why 100 check-ins?

It is a stated minimum for a stable estimate in a pilot, not a claim of statistical power. All enrolled students are invited. Below 100 I report counts and exact intervals only, framed as pilot signals.

What if few students take part?

Uptake is itself a finding for a feasibility pilot. I report the participation rate against the number enrolled, and I do not claim more than the numbers allow.

Can you show that mental health improves?

No, and I do not claim it. The pilot shows uptake, time to first contact, referrals that reach a visit, usability and trust. Clinical improvement needs a later, larger study.

How do you protect anonymity, and how can a nurse then reach a student?

Answers carry an automatic code such as STUD-0001; no name, admission, service or phone number is stored with them. If a student asks for a call-back they choose to type a safe contact detail; the nurse sees a ticket code, the urgency and only the scores shared. If nothing is shared the nurse cannot find the student, so help numbers show at once and the reply waits in the app.

What if a student is at serious risk and anonymous?

Red results show help numbers immediately and offer a same-day call-back. Contact is targeted within 8 hours in cover hours; after hours, numbers and the duty medical officer. A written duty-of-care protocol is submitted with the proposal. I state the limit honestly: anonymity means a student who shares nothing cannot be found.

Will commanders or lecturers see answers?

No. Staff see cases by code only and never individual answers outside the nurse. Dashboards and reports hide any group under 10, and totals count visible groups only. The undertaking that results are not used for marks, progression, discipline or admission is part of the letter of support.

Why ask about debt, shylocks and held ID cards?

Money strain and informal lending are a pressure on service people, and a held ID card can turn into a discipline problem. The questions are anonymous, ask for no lender name, place or service number, and every one offers Prefer not to say. Money answers never change the colour; they point to welfare help.

Why ask about contact with civilian police?

Altercations with civilian police are a reported concern and can lead to discipline or legal cases. The items are anonymous and optional, ask for no names, places or dates, and can be switched off in one setting if the ethics bodies advise. They help show whether students need legal and welfare guidance.

Why ask about betting?

Betting is a common route into debt, informal lending and held ID cards, and it affects families and work. The items are anonymous and optional: how often, two screening questions from the published Lie/Bet screen, and whether betting led to borrowing. They do not change the colour; they point to welfare and money help.

How valid are your questions?

The standard scales (PHQ-9, GAD-7, PC-PTSD-5, AUDIT-C) are used as published; PHQ-9 and GAD-7 have been validated in East Africa. Locally written items are labelled not validated and are tested with 10 to 15 students (clarity, timing, reliability) before use.

How good is the Kiswahili?

It is a draft. A translator checks it, a second person translates it back into English, and validated Kiswahili wording is used where it exists (for example the PHQ-9). The pre-test includes Kiswahili readers.

Why no Delphi, interviews or focus groups?

They are costly and not needed for a pilot. Wording is improved by a short five-staff review, student testing and open suggestion boxes.

Which approvals do you need, and in what order?

Department and supervisor, university ethics, KDF-ISERC (no fee for serving personnel researching in military establishments), DCHS permission, NACOSTI, and data protection. I confirm the order and forms with my supervisor and each body, and collect no data before they are complete.

Is it viable, and what does it cost?

About KES 154,000 including 10% contingency. Hosting is free (GitHub Pages and a free Supabase plan, or KoboToolbox). DCHS is asked for support, not hosting. The main running cost is nurse time.

Where will data be stored, and is it secure?

Practice data stay in the browser in demonstration mode; real data go to a database with role-based access, or KoboToolbox, under the approved retention rules and a data protection impact assessment. Free hosting is outside Kenya, so that is stated and reviewed with the data protection office. The system has been tested on a local test database only, not on a live server or with students.

Is the scope too ambitious for a student project?

The core is one pilot with enrolled DCHS students: check-in, weekly pulse, nurse call-back, dashboard and monthly report. Applicants, serving units, families, a comparison group and registry linkage are optional and never delay the core.

Can the result be used for admissions or selection?

No. The study advises against it, because it would conflict with Article 27 of the Constitution and the Mental Health (Amendment) Act, 2022, would make students answer to look well, and would destroy trust in the service.

What if the nurse cannot cover every hour?

Cover hours are stated, there is an after-hours route, and enrolment is paused if the rota cannot cope.

What are the study's main limitations?

Feasibility, not effect; possible socially desirable answers; small numbers; unvalidated local items; one nurse; a draft translation. Each has a stated response in the proposal.

What happens after the pilot?

A costed adoption plan and a policy brief for DCHS and the Medical Services, and a decision by them on adoption, a Kenyan-hosted copy and any extension to units and deployment.

Placeholders to complete before the defence

- Registration number and supervisor name on the cover page.
- DCHS enrolment numbers (to show an expected participation rate).
- Real contact numbers, replacing +2547xxxxxxxx.
- Cost of a lost place from the college finance office (the break-even figure is illustrative).
- Name of the ethics committee and its submission dates.
- Translator and back-translator names.